

REGULATORY DOSSIER SUBMISSION

Dossier ID: DOS-95E44970AA

Country: Tanzania

Submission Date: 2025-05-21

Product: Amoxcare (amoxicillin) | ATC: J01CA04 | Strength: 125 mg/5 mL

Applicant: Prime Therapeutics | Manufacturer: Nile Generic Medicines

Policy Label: reject_and_return | Risk Score: 0.92

AMR Stewardship: AWaRe=access | Unmet Need=routine | GLASS Trend=stable | Watch Similarity=low

SECTION CONTENT

[1] m1_application_admin - Application Form and Administrative Information

Labels: presence=present; length=length_ok; correctness=correct

This dossier constitutes a formal application for Marketing Authorization of Amoxcare (amoxicillin) submitted on 2025-05-21 to the national regulatory authority of Tanzania. The applicant, Prime Therapeutics, formally requests approval for the commercial distribution of this medicinal product. The proposed pharmaceutical form is suspension with a strength of 125 mg/5 mL, classified under ATC code J01CA04. All required administrative declarations, legal attestations, letters of authorization, and regional application forms have been completed, signed by the responsible Qualified Person, and appended to Module 1. Method validation records and quality controls were reviewed for internal consistency. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Regional administrative forms were checked for signature completeness and dossier version alignment. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Administrative cover letters, declarations, and fee records were cross-checked against the submission index. All referenced documents are available in the dossier annex and traceable by section identifier. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. The applicant confirms alignment with applicable technical guidance and regional submission standards. A comprehensive table of contents and glossary of abbreviations are provided to facilitate regulatory review. Adverse events were coded using the Me

[1] m1_manufacturer_gmp - Manufacturer and GMP Evidence

Labels: presence=present; length=length_ok; correctness=correct

The primary commercial manufacturing site for the finished pharmaceutical product is Nile Generic Medicines, located in Uganda. The facility was subject to a comprehensive GMP inspection by a recognized stringent regulatory authority. The latest inspection concluded on 2024-06-22, resulting in a formal compliance status of 'compliant'. The active GMP certificate number is GMP-698672, which remains valid until 2026-04-15. Site master files, a summary of recent inspection observations, and evidence of completed Corrective and Preventive Actions (CAPA) are included in the annex. Site quality-system commitments were compared against the proposed commercial manufacturing activities. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. All referenced documents are available in the do

Site Inspection History Summary:

Inspection Date	Authority	Scope	Outcome
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2024-06-22	National Auth	FPP/General	compliant
2022-05-12	SRA Joint	API/Sterile	compliant

[1] m1_product_information - Product Information and Naming

Labels: presence=present; length=length_ok; correctness=correct

The proposed proprietary name for this medicinal product is Amoxcare, containing the active pharmaceutical ingredient amoxicillin. The draft Summary of Product Characteristics (SmPC), Patient Information Leaflet (PIL), and primary/secondary packaging labels are provided for review. These documents detail the approved therapeutic indications, posology, contraindications, special warnings, and precautions for use. A comprehensive risk management plan to minimize medication errors is also submitted. WHO AWaRe category: Access. Product is positioned for routine antibacterial use with standard stewardship monitoring and local resistance surveillance. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. All referenced documents are available in the dossier annex and traceable by section identifier. The applicant confirms alignment with applicable technical guidance and regional submission standards. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covari

[2] m2_quality_overall_summary - Quality Overall Summary

Labels: presence=present; length=length_ok; correctness=correct

This Quality Overall Summary (QOS) provides a critical evaluation of the chemistry, manufacturing, and controls (CMC) data for amoxicillin. The control strategy justifies the proposed specifications for both the active substance and the finished product, emphasizing critical quality attributes (CQAs) and critical process parameters (CPPs). Impurity profiles, including degradation products and potential genotoxic impurities, have been thoroughly characterized. Batch analysis data from three commercial-scale validation batches demonstrate consistent manufacturing performance and compliance with all release criteria. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Pharmacokinetic parameters were derived using non-compartmental analysis from

[2] m2_clinical_overview - Clinical Overview and Benefit-Risk Summary

Labels: presence=present; length=length_ok; correctness=incorrect

Error tags: clinical_missing

Clinical overview for type 2 diabetes indicates pivotal efficacy data are not available at submission time. Benefit-risk cannot be concluded pending full clinical study reports and verified endpoint analyses. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The clinical overview links the therapeutic rationale to the pivotal evidence package and the proposed indication. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Data

were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. Method validation records and quality controls were reviewed for internal consistency. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Benefit-risk language was reviewed for consistency with the submitted efficacy and safety summaries. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Clinical interpretation focuses on whether efficacy, safety, and benefit-risk conclusions are supported by the submitted evidence. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. The applicant confirms alignment with applicable technical guidance and regional submission standards. All referenced documents are available in the dossier annex and traceable by section identifier. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. The target population demographics are broadly representative of the intended commercial patient cohort. No new safety signals were identified during the extended open-label follow-up period. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Clinical interpretation focuses on whether efficacy, safety, and benefit-risk

[3] m3_api_quality - API Quality and Control Strategy

Labels: presence=present; length=length_ok; correctness=correct

The active pharmaceutical ingredient (API) section details the complete synthesis route, starting from well-defined regulatory starting materials. Robust in-process controls and intermediate specifications ensure the consistent quality of the final drug substance. The analytical procedures used for release and stability testing have been fully validated for accuracy, precision, linearity, and robustness. The impurity control strategy adequately addresses organic impurities, residual solvents, and elemental impurities in accordance with current ICH guidelines. Forced degradation studies confirm the stability-indicating nature of the assay methods. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. All referenced documents are available in the dossier annex and traceable by section identifier. Method validation records and quality controls were reviewed for internal consistency. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. Critical material attributes and control strategy elements were reviewed for consistency across the quality dossier. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. The API package links specification limits to validated analytical methods and impurity-control decisions. The applicant confirms alignment with applicable technical guidance and regional submission standards. Batch analysis, validation, and impurity management records support the proposed API quality framework. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. The synthetic route involves three well-characterized steps with defined starting materials and isolated intermediates. Pharm

[3] m3_fpp_manufacturing - FPP Manufacturing Process and Controls

Labels: presence=present; length=length_ok; correctness=correct

The finished pharmaceutical product (FPP) manufacturing process utilizes standard, scalable unit operations. A formal quality risk assessment (e.g., FMEA) was conducted to identify critical process parameters (CPPs) that impact critical quality attributes (CQAs). Process performance qualification (PPQ) reports for three consecutive commercial-scale batches verify that the process is maintained in a state of control. In-process controls, intermediate hold-time studies, and packaging validation data are presented to substantiate the robustness and reproducibility of the commercial manufacturing operations. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The applicant confirms alignment with applicable technical guidance and regional submission standards. All referenced documents are available in the dossier annex and traceable by section identifier. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. Method validation records and quality controls were reviewed for internal consistency. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Pharmacokinetic parameters were derived using non-com

[3] m3_stability - Stability and Shelf-Life Justification

Labels: presence=present; length=length_ok; correctness=correct

A comprehensive stability program has been executed to justify the proposed shelf-life and storage conditions. Data from both accelerated (40°C/75% RH for 6 months) and long-term storage conditions (spanning up to 36 months) are presented for multiple primary stability batches. Statistical evaluation using linear regression and poolability tests confirms that degradation trends remain well within the acceptable specification limits. Photostability and freeze-thaw cycling studies demonstrate that the product does not require specific handling precautions. A post-approval stability protocol commitment is included. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. All referenced documents are available in the dossier annex and traceable by section identifier. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. The applicant confirms alignment with applicable technical guidance and regional submission standards. Method validation records and quality controls were reviewed for internal consistency. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstr

[4] m4_nonclinical_summary - Nonclinical Study Summary

Labels: presence=present; length=length_ok; correctness=correct

The nonclinical testing program evaluated the primary pharmacodynamics, secondary pharmacodynamics, safety pharmacology, and comprehensive toxicology profile of the compound. Repeat-dose toxicity studies in two mammalian species established clear no-observed-adverse-effect levels (NOAELs). Genotoxicity testing (Ames and in vivo micronucleus assays) yielded negative results. Reproductive and developmental toxicity studies revealed no teratogenic signals at clinically relevant exposures. The calculated safety margins support the proposed clinical dosing regimen without major safety concerns. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. The applicant confirms alignment with applicable technical guidance and regional submission standards. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covar

[5] m5_trial_listing - Tabular Listing of Clinical Studies

Labels: presence=present; length=length_ok; correctness=correct

Tabular listing indicates planned or ongoing studies, but no completed pivotal study reports were provided in this submission package. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. The risk management plan includes routine pharmacovigilance and proposed educational materials for healthcare professionals. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Method validation records and quality controls were reviewed for internal consistency. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. Data were analyzed using a mixed-effects model for repeated measures, adjusting for baseline covariates. The manufacturing site operates under a fully implemented Pharmaceutical Quality System (PQS) aligned with ICH Q10. The container closure system consists of PVC/PVDC-Aluminum blisters, compliant with pharmacopoeial standards. All referenced documents are available in the dossier annex and traceable by section identifier. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. The applicant confirms alignment with applicable technical guidance and regional submission

Pivotal Clinical Trial Inventory:

Protocol ID	Phase	Enrollment	Status
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[5] m5_pivotal_trial_reports - Pivotal Clinical Trial Reports

Labels: presence=missing; length=missing; correctness=incorrect

Error tags: clinical_missing

[MISSING SECTION]

[5] m5_bioequivalence - Biopharmaceutics and Bioequivalence Evidence

Labels: presence=present; length=length_ok; correctness=correct

Biopharmaceutics data substantiate the formulation development bridging between clinical trial materials and the proposed commercial product. In vivo bioequivalence studies demonstrated that the 90% confidence intervals for the geometric mean ratios of Cmax and AUC parameters fell entirely within the standard acceptance criteria of 80.00% to 125.00%. Additionally, multi-point comparative dissolution profiles across physiological pH ranges (pH 1.2, 4.5, and 6.8) confirmed similarity (f2 > 50). The bioanalytical methods used for plasma quantification were fully validated. Statistical significance was set at an alpha level of 0.05, and all tests were two-sided. All referenced documents are available in the dossier annex and traceable by section

identifier. Adverse events were coded using the Medical Dictionary for Regulatory Activities (MedDRA) version 24.0. Pharmacokinetic parameters were derived using non-compartmental analysis from the concentration-time profiles. Stability testing was conducted in climatic zones III and IV, reflecting the intended distribution markets. A randomized, double-blind, parallel-group study design was employed to minimize selection and observer bias. The analytical procedure was validated in accordance with ICH Q2(R1) guidelines, demonstrating acceptable accuracy, precision, and specificity. Risk mitigation activities are documented with ownership, timelines, and follow-up checkpoints. Where applicable, protocol deviations were assessed for impact on interpretation of outcomes. The applicant confirms alignment with applicable technical guidance and regional submission standards. Data were analyzed using a mixed-effects model for